

Positive Behaviour Support & Restrictive Practice Policy

SECTIONS 1–3: PURPOSE, SCOPE & PRINCIPLES

1 PURPOSE

To promote a culture of positive behaviour support (PBS) and ensure any use of restrictive practice is justified, proportionate, and in line with human rights principles, the Health Act 2007, and HIQA's PBS Guidance (2023).

- ✓ Promote dignity, independence, and respectful interactions
- ✓ Prevent and reduce behaviours of concern through proactive, person-centred approaches
- ✓ Ensure restrictive practices are only used as a last resort

2 SCOPE

- ✓ All staff (permanent, agency, part-time, voluntary) and all residents
- ✓ All settings where staff interact with residents, on-site or off-site
- ✓ Read in conjunction with Safeguarding Policy, HIQA Standards, and PBS Guidance 2023

3 PRINCIPLES

- ✓ **Person-Centred:** Support plans informed by individual strengths, preferences, needs, and communication style
- ✓ **Least Restrictive:** Use the least restrictive intervention possible; only as last resort when immediate risk exists
- ✓ **Human Rights & Consent:** Right to freedom, dignity, bodily autonomy; any restriction must be legally compliant, time-limited, documented
- ✓ **Prevention First:** Understand causes of behaviours; implement proactive supports including triggers, cues, and de-escalation
- ✓ **Transparency:** All practices subject to governance, oversight, audit; families and regulators involved in reviews

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SECTIONS 4-5: PBS FRAMEWORK & RESTRICTIVE PRACTICES

4 POSITIVE BEHAVIOUR SUPPORT (PBS)

Goals:

- ✓ Promote independence, dignity, and participation; reduce frequency/severity of behaviours of concern
- ✓ Improve communication and emotional regulation; prevent need for restrictive interventions

Key Components:

- ✓ **Functional Behaviour Assessment (FBA):** Understanding cause and purpose of behaviours
- ✓ **Behaviour Support Plans (BSPs):** Triggers, early warning signs, positive strategies, de-escalation, staff roles
- ✓ **Skill-building:** Helping residents develop alternative behaviours meeting their needs
- ✓ **Environment adaptations:** Changes to reduce distress or overstimulation
- ✓ **Collaboration:** Planning involves resident, family, keyworkers, clinicians, and external specialists

5 RESTRICTIVE PRACTICES

Categories (per HIQA):

- ✓ **Physical Restraint:** Bodily force to restrict movement
- ✓ **Mechanical Restraint:** Devices restricting free movement (locked lap belts, mittens)
- ✓ **Environmental Restraint:** Restricting access to areas, items, or exits
- ✓ **Chemical Restraint:** Medication to manage behaviour, not treat diagnosed condition
- ✓ **Seclusion:** Isolating resident where they cannot leave freely
- ✓ **Psychological/Relational:** Intimidation, threats, or power dynamics to coerce

All must be proportionate, time-limited, legally compliant, used only as last resort. Every instance logged in the Restrictive Practice Register and reviewed by PIC.

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SECTIONS 6–8: GOVERNANCE, TRAINING & OVERSIGHT

6 GOVERNANCE

- ✓ **PIC:** Reports, reviews, documents all restrictive interventions; signs off; audits logs and BSPs
- ✓ **Senior Management:** Reviews data monthly for trends; allocates resources for training and PBS
- ✓ **Behaviour Specialist:** Develops/updates BSPs; trains staff; participates in post-incident reviews

Restrictive Practice Register records:

- ✓ Type, date/time/duration, reason, staff involved, resident's response, review outcome/actions
- ✓ Reviewed monthly by PIC, quarterly by senior management

7 STAFF TRAINING

- ✓ **Mandatory:** PBS, de-escalation/crisis intervention, restrictive practices, safeguarding, human rights/consent, HIQA standards
- ✓ **Refresher:** Every 2 years or more frequently as required
- ✓ **Competency:** Assessed through supervision, observation, and post-incident reviews
- ✓ **Specialist:** External professionals for complex behavioural needs; aligned with Health Act 2007

8 OVERSIGHT

- ✓ **Oversight Group:** Multidisciplinary, meets quarterly — reviews trends, compliance, BSP effectiveness, reduction opportunities
- ✓ **Quarterly Reports:** Number/type of interventions, outcomes, de-escalation success/failure, actions to reduce future use
- ✓ **HIQA:** Notifiable practices submitted via portal; internal audits twice yearly checking logs, justifications, debriefs

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SECTIONS 9–10: CONSENT & REVIEW

9 CONSENT

9.1 Informed Consent:

- ✓ Information provided in accessible format (easy-read, visuals)
- ✓ Staff explain nature, purpose, risks, restrictions, and right to refuse
- ✓ Consent freely given, specific, and recorded in care/support plan

9.2 Capacity & Supported Decision-Making:

- ✓ Where capacity lacking, Assisted Decision-Making (Capacity) Act 2015 applies
- ✓ Decision-Making Representative, family, or advocate involved
- ✓ All decisions in resident's best interest; legally justified and documented

9.3 Withdrawal of Consent:

- ✓ Consent can be withdrawn at any time
- ✓ Respected immediately where safe to do so; reviewed by PIC; documented in Register

10 POLICY REVIEW & CONTINUOUS IMPROVEMENT

- ✓ Reviewed annually by PIC and Registered Provider; earlier if legislation, HIQA guidance, or audit findings change
- ✓ Residents, advocates, and staff consulted on changes
- ✓ Trends and learnings from audits feed into training plans, BSPs, and organisational risk management